

HEALTHCARE COMPLIANCE · REFERENCE

Medical Debt Collection Data Compliance

Navigating the new state credit-reporting laws — effective September 2025

As of September 2025, the credit bureaus changed how identifiable medical debt data is displayed for consumers in a number of states. This paper summarizes the regulatory timeline, the operational impact on healthcare providers and revenue-cycle-management (RCM) teams, and the practical steps to stay compliant while protecting legitimate receivables.

Key takeaways

- September 19, 2025:** Delaware (SB 156), Maine (LD 558), Maryland (HB 1010), Minnesota (SB 4097) and Oregon (SB 506) moved to hidden identifiable medical debt data (approx. 3% of displayable trades).
- September 26, 2025:** Connecticut (SB 395) and Virginia (HB 1370) adopted the same display restrictions (approx. 1% of displayable trades).
- Earlier implementations:** Colorado (Aug 6, 2023); New York (Dec 13, 2023); California, Illinois and Rhode Island (Dec 20, 2024); Vermont and Washington (Jun 27, 2025).
- Credit reporting remains only one tool; display restrictions require adjusted Metro 2 mapping, dispute handling and updated SOPs.
- Align vendors and staff on timing; maintain audit trails and written confirmations from the bureaus.

Effective dates and state laws

DATE	STATES	LAW REFERENCES	EST. IMPACT
Sept 19, 2025	Delaware, Maine, Maryland, Minnesota, Oregon	DE SB 156; ME LD 558; MD HB 1010; MN SB 4097; OR SB 506	Approx. 3%
Sept 26, 2025	Connecticut, Virginia	CT SB 395; VA HB 1370	Approx. 1%

Operational impact for providers and RCM teams

Display restrictions reduce downstream credit-score effects and require adjustments across reporting workflows, pre-legal review and dispute resolution. Providers should ensure accurate residency determination, correct application of state-specific logic, and updated patient communications.

Immediate actions to take

- Reconcile data-furnisher settings with each state's display rules and cutover dates.
- Update SOPs for disputes, verifications and Metro 2 mapping; retrain frontline staff.
- Document bureau acknowledgments of logic changes; maintain audit trails.
- Refresh patient financial-assistance materials and communications to reflect the new rules.

How Southwest Recovery de-risks the transition

- State-by-state rule logic maintained with version control and audit history.
- Compliance-first quality gates before any reporting or escalation, with transparent client dashboards.
- Skip-tracing and data validation to keep contact and residency data accurate.
- Client training resources and checklists aligned to each go-live date.

Appendix A — statute & implementation notes

- State cohorts are based on consumer residency; ensure residency determination and updates are reliable and documented.
- These rules affect the *display* of identifiable medical debt data; obligations remain collectible as permitted by law.
- Maintain written confirmations from bureaus and vendors regarding logic changes and timing.
- Reassess adverse-action language and patient communications to avoid misleading statements.

Talk with an SWRS compliance specialist

We keep healthcare clients current as federal and state medical-debt rules evolve — and work every account inside that framework.

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